



COMMUNITY ONCOLOGY ALLIANCE

Dedicated to Advocating for Community Oncology Patients and Practices

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February 23, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy and the Office of the National Coordinator
for Health Information Technology
Department of Health and Human Services
7033A, 330 C Street SW
Washington, D.C. 20201

Submitted electronically via regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane:

On behalf of the Board of Directors of the Community Oncology Alliance (“COA”), we are submitting this comment letter regarding the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (“RFI”).

COA is an organization dedicated to advocating for the complex care and access needs of patients with cancer and the community oncology practices that serve them. COA is the only nonprofit organization in the United States dedicated solely to independent community oncology practices, which serve the majority of Americans receiving treatment for cancer. Since its grassroots founding close to 25 years ago, COA’s mission has been to ensure that patients with cancer receive quality, affordable, and accessible care in their own communities where they live and work.

COA is committed to providing solutions to the challenges that community oncology patients and physicians face throughout the care journey. We seek to ensure that cancer care is delivered with the utmost quality and in an accessible and affordable manner.

Recognizing the role artificial intelligence (“AI”) plays in clinical care and the ongoing momentum around its use, COA has established an AI and Digital Transformation Task Force. Our feedback and recommendations below are informed by the community oncologists and practice leaders who are actively adopting AI within their practices.

Background and Summary of Feedback and Recommendations

Independent community oncology practices are the backbone of cancer care delivery in the United States, caring for most patients close to home across urban, suburban, rural, and underserved communities. Because this is where the majority of treatment decisions and longitudinal follow-up occur, supporting digital transformation in community settings is essential to achieving national goals for quality, health outcomes, access, and affordability in cancer care.¹

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¹ Hughes-Halbert, C. National Cancer Institute. “Bringing Research to the Community to Reduce Cancer Disparities.” 21 September 2017. Available [Here](#).

The digital transformation required to integrate AI into clinical care is substantial, necessitating not only financial investment but also significant time and human resources. Implementing AI solutions involves a steep learning curve and the adaptation of existing workflows, which can overwhelm small practices with limited personnel and financial bandwidth. Without proper incentives, the risks may outweigh the potential benefits, making it especially challenging for community practices to engage in this essential evolution.

Historically, the introduction of electronic health records (“EHRs”) provides a fitting precedent. Incentives in that case enabled practices to invest in the necessary technology and training while offsetting the initial transition costs. To facilitate the adoption of AI tools, we urge the Department of Health and Human Services (“HHS”) to consider a similar incentive structure. This could include financial reimbursement models for implementing AI applications that drive improved patient outcomes, efficiency, and cost-effectiveness. By promoting access to high-value AI clinical interventions, we can ensure that community practices remain viable and do not face pressures to consolidate into higher-cost hospital service settings.

Failing to effectively transition into a digital-first approach could jeopardize the sustainability of smaller practices. As pressures mount from larger health care systems that can leverage advanced technologies to reduce costs and enhance patient care, the risk of community oncology practices falling behind is real. This would ultimately lead to reduced patient choice and increased health care costs for the communities we serve.

Our responses focus on questions 1, 2, 6, and 9 in the RFI. Next are our summary responses, followed by more detailed responses.

Question 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care include financial barriers and resource constraints; uncertainty regarding regulatory expectations, accountability, and legal liability for non-device AI tools; and limited access to interoperable data across care settings.

1. Financial Barriers and Resource Constraints

Up-front and non-recoverable implementation costs may limit practices’ adoption of AI tools and, once implemented, a lack of reimbursement for AI-enabled clinical or administrative functions places pressure on practices’ long-term and sustained AI use. For practices to fully leverage AI in clinical care, the financial incentives must account for the full range of costs associated with AI use, including additional investment in technical infrastructure and expertise. Even when AI tools are clinically promising, practices may be unable to deploy them safely or effectively without significant external support. This creates a widening gap between large, well-resourced health systems and community-based practices, potentially exacerbating disparities in access to innovation.

2. Uncertainty Regarding Regulatory Expectations, Accountability, and Legal Liability

For providers, most professional liability policies were not designed with AI-assisted clinical workflows in mind. Providers must also be able to understand the limitations, training data sources, and intended use of AI systems – providers cannot reasonably assess risk or ensure appropriate clinical oversight without transparency.

Malpractice risk is another concern, as providers lack clarity on whether the use of AI tools may affect malpractice coverage or exposure, particularly when tools influence documentation, clinical reasoning, or treatment decisions. On the vendor side, many AI vendors offer limited service-level agreements (“SLAs”)

related to model performance, uptime, data provenance, or error rates. This creates a mismatch between the clinical risk assumed by providers and the contractual protections offered by vendors.

3. **Limited Access to High-Quality, Interoperable, and Clinically Usable Data**

Even where interoperability standards exist, such as those promulgated through the Health Data, Technology, and Interoperability rules, access to usable, high-quality data remains a major barrier to effective AI deployment. Clinical data across care settings is often incomplete, inconsistently structured, poorly normalized, and fragmented across multiple systems, vendors, and care sites. Practices face challenges related to data quality and heterogeneity, interoperability gaps, context loss across care settings, and data governance and access restrictions.

Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

HHS should consider several strategies to incentivize the adoption of AI and support digital transformation in clinical care. We recognize that congressional action may be required to enable HHS to set up these initiatives. However, our hope is that HHS will adopt these initiatives whenever possible. COA recommends a multi-faceted approach that includes:

1. **Financial Incentives and Infrastructure Development**

- i. **Grants and Funding Opportunities:** Establishment of grant programs specifically for community practices to fund AI implementation projects and require transparent reporting of safety, performance, and health outcomes impact.
- ii. **Tax Incentives:** Working with the Internal Revenue Service (“IRS”), HHS should provide tax incentives for qualifying AI technologies and investments in digital infrastructure.
- iii. **Pilot Programs:** Launch a pilot program to incentivize early adopters of AI technologies, including through the Center for Medicare & Medicaid Services (“CMS”) Innovation Center Authority at Social Security Act (“SSA”) § 1115A.

2. **Training and Partnerships**

- i. **Data Sharing Initiatives:** Enhanced interoperability standards and government-led initiatives to encourage data sharing among practices and AI systems would make it easier to demonstrate the efficacy of AI tools through shared insights and benchmarks. CMS could also leverage the voluntary CMS Health Technology Ecosystem pledges to further accelerate AI adoption.
- ii. **Recognition Programs:** Establish awards or recognition for practices that successfully implement AI-driven solutions, boosting visibility in the community and providing positive reinforcement for innovative efforts. HHS’s Million Hearts Hospitals & Health Systems Recognition Program could be used as a model, and the Health Technology Ecosystem pledges could include an AI-specific pledge.
- iii. **Public-Private Partnerships:** Collaborations between government entities and private sector technology developers could create tailored AI solutions for small practices. This could also include shared investment in research and development. Partnerships should be paired with shared implementation playbooks, model contracts, and standardized governance requirements to reduce

procurement friction and duplication across practices. Elements of CMS’s Advancing Chronic Care with Effective, Scalable Solutions (“ACCESS”) Model and Make America Healthy Again: Enhancing Lifestyle and Evaluating Value-Based Approaches Through Evidence (“MAHA ELEVATE”) Model could be used to better expand access to AI-supported oncology care, as could partnerships with, and voluntary commitments from, private organizations.

- iv. **Training and Education Programs:** Develop or subsidize training programs for health care providers to improve competency in using AI tools, with CMS serving as a resource repository for sharing best practices. We also recommend CMS develop a centralized AI initiatives webpage, with all internal and external CMS initiatives related to AI, including HHS-provided training and education programs.

3. **Regulatory Streamlining**

HHS should simplify the regulatory process for new AI tools, ensuring faster approval and easier integration into practices. At the same time, health care providers must be able to adopt tools without ambiguity, and there must be consistency, enforcement, and accountability. HHS guidance for both scenarios will accelerate AI adoption.

4. **Reimbursement**

- i. **Performance-Based Reimbursement Models:** Adequate, sustained reimbursement is needed to support the longevity of AI adoption in clinical care. Per-member-per-month payments, shared-savings attribution within value-based programs, bonus points towards the Promoting Interoperability performance category, or final score in the Merit-Based Incentive Payment System (“MIPS”) program, would provide a further incentive for investment in AI. Current CMS models, such as the ACCESS Model, could also be leveraged to reward practices for successfully using AI to improve outcomes while allowing for flexibility in the tools used.
- ii. **Ongoing Reimbursement for and Valuation of AI Tools:** AI tools must be adequately reimbursed, and reimbursement must keep pace in a rapidly changing environment, with CMS’s reimbursement methodology for AI tools under Medicare, via the Physician Fee Schedule (“PFS”), being particularly impactful.² As new AI tools are being developed and existing tools continue to be updated, the underlying payment methodology for AI tools may need to be reevaluated more frequently, and methodologies should consider the differences in AI tools compared to many of the services CMS currently reimburses.³ Providers also need updated codes to submit claims for AI tools.

Question 6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Community oncologists are using AI tools in many ways, including clinical decision-making support and to reduce administrative burden. AI applications also include cancer detection, screening, diagnosis, tumor analysis, biomarker assessment, and drug discovery.⁴ However, AI pilot programs conducted by community oncology practices have found that: 1) data quality and preparation remain significant barriers; 2) operational lift is frequently underestimated;

² Longyear R and Berenson R. “Artificial Intelligence Payment Policies: Challenges For CMS And The Medicare Physician Fee Schedule.” *Health Affairs*. January 2026. Available [Here](#).

³ Ibid.

⁴ Luchini C, et al. “Artificial intelligence in oncology: current applications and future perspectives.” *British Journal of Cancer*. 26 November 2021. Available [Here](#).

3) quality-of-life improvements do not always translate into financial return on investment (“ROI”); 4) voice and patient-facing AI tools remain immature; and 5) revenue cycle applications show promise but face trust barriers.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers generally support the use of technology that improves access, reduces delays, and enhances coordination of care. At the same time, they value transparency, strong privacy protections, and continued human clinical oversight. Policies that reinforce these expectations will be essential to sustaining trust in the use of AI in clinical care. COA also has serious concerns regarding the use of AI in utilization management and the impact on patients. We have long expressed concerns about how PA harms patients, and introducing AI into the process poses even greater risks for patients.^{5,6} Providers and patients are often not aware that AI is involved in utilization review, with one survey finding that less than a quarter of insurers tell providers when AI is used.⁷

Following are further detailed responses to questions 1,2, 6, and 9.

Question 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

There are several practical barriers that limit the adoption of AI in clinical care, including financial barriers and resource constraints, uncertainty regarding regulatory expectations and accountability for non-device AI tools, and limited access to interoperable data across care settings. Without targeted federal guidance, incentives, and technical assistance, AI adoption risks becoming concentrated in only the largest health systems – undermining broader policy goals of scalable innovation in health care delivery.

1. Financial Barriers and Resource Constraints

Financial barriers are a significant hurdle to the adoption of AI in clinical care. Up-front and non-recoverable implementation costs may limit practices’ adoption of AI tools, and once implemented, a lack of reimbursement for AI-enabled clinical or administrative functions places pressure on practices’ long-term and sustained AI use. For practices to fully leverage AI in clinical care, the financial incentives must account for the full range of costs associated with AI use.

Limited technical infrastructure and expertise at the practice level are critical and often overlooked barriers, particularly within small and mid-sized community oncology groups. Many practices lack the internal technical capacity needed to implement, validate, and govern AI tools responsibly. Many practices lack dedicated data engineering or informatics staff; AI governance or model evaluation frameworks; infrastructure to monitor performance, bias, or drift over time; resources to conduct vendor due diligence or technical validation; and expertise to integrate AI tools safely into clinical workflows. As a result, even when AI tools are clinically promising, practices may be unable to deploy them safely or effectively without significant external support. This creates a widening gap between large, well-resourced health systems and community-based practices, potentially exacerbating disparities in access to innovation.

2. Uncertainty Regarding Regulatory Expectations, Accountability, and Legal Liability

⁵ Community Oncology Alliance. “COA Comments on WISer Model.” 14 August 2025. Available [Here](#).

⁶ Community Oncology Alliance. “COA Comments on 2027 Medicare Advantage and Part D Proposed Rule.” 23 January 2026. Available [Here](#).

⁷ DeFraim K et al. “Health Insurance Artificial Intelligence/Machine Learning Survey Results.” National Association of Insurance Commissioners (NAIC). 2025. Available [Here](#).

Uncertainty regarding regulatory expectations, accountability, and legal liability for non-device AI tools used in clinical workflows is a major barrier to adoption. While the FDA has established regulatory pathways for Software as a Medical Device (“SaMD”), many AI tools currently being deployed in health care – such as clinical decision support, ambient documentation, summarization tools, and workflow automation – fall outside traditional medical device classifications. While the FDA recently released updated Clinical Decision Support Software – Guidance for Industry and FDA Staff, uncertainty remains for providers in several critical areas⁸:

- i. **Accountability and liability:** It remains unclear how legal responsibility is apportioned among clinicians, health systems, and vendors when AI tools contribute to clinical decision-making errors. This uncertainty is particularly concerning given the potential for AI hallucinations, data bias, or incomplete context in large language models.
- ii. **Malpractice risk:** Most professional liability policies were not designed with AI-assisted clinical workflows in mind. Providers lack clarity on whether use of AI tools may affect malpractice coverage or exposure, particularly when tools influence documentation, clinical reasoning, or treatment decisions.
- iii. **Vendor accountability and transparency:** Many AI vendors offer limited SLAs related to model performance, uptime, data provenance, or error rates. This creates a mismatch between the clinical risk assumed by providers and the contractual protections offered by vendors.
- iv. **Trust and defensibility:** Clinicians must understand the limitations, training data sources, and intended use of AI systems. Without transparency into model behavior and performance characteristics, providers cannot reasonably assess risk or ensure appropriate clinical oversight. The potential for bias, hallucinations, and errors underscores the need for transparency about the underlying assumptions of AI tools. Ultimately, if a data set being used in an AI model does not match a patient, it introduces biases and limits the applicability of the tool and findings to the patient.⁹

3. **Limited Access to High-Quality, Interoperable, and Clinically Usable Data**

Limited access to high-quality, interoperable, and clinically usable data across care settings is another barrier to AI adoption. Even where interoperability standards exist, such as those promulgated through the Health Data, Technology, and Interoperability rules, access to usable, high-quality data remains a major barrier to effective AI deployment. Clinical data across care settings is often incomplete, inconsistently structured, poorly normalized, and fragmented across multiple systems, vendors, and care sites. Key challenges include:

- i. **Data quality and heterogeneity:** Real-world clinical data frequently contain missing values, inconsistent coding, unstructured text, and variable documentation practices. AI models trained on such data may produce unreliable or biased outputs.
- ii. **Interoperability gaps:** While standards such as Fast Healthcare Interoperability Resources (“FHIR”) have improved data exchange, real-world implementation remains inconsistent. Many data elements critical for oncology care – such as staging, biomarker data, treatment intent, or outcomes – are not reliably structured or exchanged.

⁸ Food & Drug Administration. “Clinical Decision Support Software. Guidance for Industry and Food and Drug Administration Staff.” 29 January 2026. Available [Here](#).

⁹ Patt D. “How AI Is Reshaping Oncology Practice: Insights From ASCO’s AI Task Force.” 12 June 2025. Available [Here](#).

- iii. **Context loss across care settings:** Cancer care frequently spans health systems, laboratories, imaging centers, and specialty providers. AI tools often lack access to a longitudinal, unified patient record, limiting their clinical utility.
- iv. **Data governance and access restrictions:** Privacy regulations, contractual limitations, and institutional risk concerns frequently restrict data sharing, even when legally permissible, further limiting the data available for AI development and validation.

Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

HHS should consider several strategies to incentivize the adoption of AI and support digital transformation in clinical care. We recommend a multifaceted approach that focuses on financial incentives and infrastructure development, training and partnerships, regulatory streamlining, and reimbursement. We recognize that congressional action may be required to enable HHS to stand up some of these initiatives. However, our hope is that HHS will adopt these initiatives whenever possible.

1. Financial Incentives and Infrastructure Development

- i. **Grants and Funding Opportunities:** We recommend that HHS establish grant programs specifically for community practices to fund AI implementation projects, including funding non-recoverable start-up costs (integration, data governance, model validation/bias testing, cybersecurity), and require transparent reporting of safety, performance, and health outcomes.
- ii. **Tax Incentives:** Working with the IRS, HHS should provide refundable tax credits, deductions, or accelerated depreciation for qualifying AI technologies and investments in digital infrastructure (including cybersecurity and interoperability upgrades), helping lower the financial burden of implementation, particularly for smaller independent practices.
- iii. **Pilot Programs:** We support the launch of pilot programs that incentivize early adopters of AI technologies, allowing practices to test AI applications in a controlled environment while providing funding and support. These could be conducted by CMS using its CMS Innovation Center Authority at SSA § 1115A – we provide specific examples in other sections of this letter below.

2. Training and Partnerships

- i. **Data Sharing Initiatives:** Enhanced interoperability standards and government-led initiatives to encourage data sharing among practices and AI systems would make it easier to demonstrate the efficacy of AI tools through shared insights and benchmarks. This includes alignment with existing interoperability requirements under 45 CFR Part 171.

The CMS Health Technology Ecosystem and its call for the health care industry to voluntarily align around a shared framework for data and access is one step towards this initiative. Many companies have pledged to deliver results in the first quarter of this year, and we look forward to seeing how those results positively impact patients. To further accelerate AI adoption in clinical care, CMS could develop a pledge category that recognizes organizations leveraging AI tools and promotes the sharing of best practices across these

organizations.¹⁰ As with the rest of the pledges, CMS could implement this effort without needing to introduce new regulations.

- ii. **Recognition Programs:** We recommend HHS establish awards or recognition for practices that successfully implement AI-driven solutions, boosting visibility in the community and providing positive reinforcement for innovative efforts. HHS’s Million Hearts Hospitals & Health Systems Recognition Program may serve as a framework for an AI recognition program that facilitates excellence in AI-supported care and innovation and could include other specialties in addition to oncology.¹¹ The Health Technology Ecosystem pledges we referenced above would also increase visibility into organizations driving the adoption of AI in clinical care.
- iii. **Public-Private Partnerships:** Collaborations between government entities and private sector technology developers could create tailored AI solutions for small practices. This could also include shared investment in research and development. Partnerships should be paired with shared implementation playbooks, model contracts, and standardized governance requirements (privacy, accountability, auditability) to reduce procurement friction and duplication across practices.

CMS’s ACCESS Model is a step towards connecting beneficiaries with technology-enabled care. As CMS may consider additional tracks or conditions in the future, we hope to see greater opportunities for oncologists and patients receiving cancer treatment. Similarly, we recommend that the MAHA ELEVATE Model’s structure of funding cooperative agreements, with the goal of complementing or augmenting care, be leveraged in the future to better expand access to AI-supported oncology care.

The Health Technology Ecosystem voluntary commitments from private companies are another example of CMS leveraging public-private partnerships, and we believe partnerships and voluntary commitments could be expanded to include voluntary financial commitments to drive AI in clinical care. For example, most recently, CMS announced that 10 health technology companies voluntarily pledged to support states in preparing to implement new Medicaid community engagement requirements. Per CMS, the companies will offer states more than \$600 million in no-cost and significantly discounted technology products and services.¹² In 2019, CMS also led the Artificial Intelligence Health Outcomes Challenge, using its CMS Innovation Center Authority at SSA § 1115A, and partnered with the American Academy of Family Physicians (“AAFP”) and Laura and John Arnold Foundation (now Arnold Ventures) to award up to \$1.65 million to challenge participants.¹³

- iv. **Training and Education Programs:** HHS should develop or subsidize training programs for health care providers to improve competency in using AI tools. This could include online courses, workshops, and on-site training to ensure practitioners feel confident in adopting new technologies, with HHS serving as a repository for sharing best practices. Role-based training for the different individuals involved in AI implementation (physicians, nurses, pharmacists, navigators, and billing), as well as change-management support, would also be beneficial.

As HHS continues to work to accelerate AI adoption in clinical care, we recommend that CMS develop an AI initiatives webpage that provides a centralized location for all internal and external CMS initiatives related to AI. While the agency has some webpages that outline the agency’s AI focuses, a central page would streamline the process for patients, providers, and companies to understand where there are

¹⁰ Centers for Medicare & Medicaid Services. “Early Adopters and All Pledges.” Available [Here](#).

¹¹ Department of Health and Human Services. “Million Hearts® Hospitals & Health Systems Recognition Program”. Available [Here](#).

¹² Centers for Medicare & Medicaid Services. “Fact Sheet: Pledges from Medicaid Technology Companies to Support Community Engagement Implementation and Related Medicaid System Improvements.” 29 January 2026. Available [Here](#).

¹³ Centers for Medicare & Medicaid Services. “CMS Artificial Intelligence Health Outcomes Challenge.” 10 June 2019. Available [Here](#).

opportunities for them to get involved in new initiatives.^{14,15} This webpage could also contain HHS-provided training and education programs, similar to how the Quality Payment Program (“QPP”) Resource Library functions.¹⁶

3. **Regulatory Streamlining**

- i. **Regulatory Streamlining:** Simplify the regulatory process for new AI tools, ensuring faster approval and easier integration into practices. Establishing clear guidelines for safe AI use will encourage more providers to adopt these technologies. This also includes clearer expectations for non-device AI governance and HIPAA-aligned privacy protections under 45 CFR Part 164.

Health care providers must be able to adopt tools without ambiguity, and there must be consistent and clear enforcement and accountability. HHS intends to establish “guidelines that maximize the potential benefits of use of AI in research and discovery and sustain trustworthiness of tools by minimizing security and privacy risks (e.g., for personal health data).”¹⁷ We recommend that HHS guidelines also expand beyond research and discovery, to when these tools are implemented and developers provide ongoing updates.

4. **Reimbursement**

- i. **Performance-Based Reimbursement Models:** Adequate, sustained reimbursement is needed to support the longevity of AI adoption in clinical care. CMS has multiple levers to provide reimbursement for AI-supported care. We suggest CMS:
 - Create reimbursement models that reward practices for successfully integrating AI to improve patient outcomes, aligned with guideline-concordant care, patient satisfaction scores, and operational efficiencies, such as reduced administrative burden and delays in access to care. As we noted above, expanding the ACCESS Model to include oncology-related clinical tracks would provide community oncologists with greater flexibility and choice in implementing technology-supported care, though financial incentives need to align with practices’ costs in expanding their use of AI-supported care. Currently, ACCESS Model tracks are limited to the following chronic conditions: Early-Cardio-Kidney-Metabolic (“eCKM”), Cardio-Kidney-Metabolic (“CKM”), Musculoskeletal (“MSK”), and Behavioral Health (“BH”).¹⁸ Initiatives could also be aligned with existing quality and interoperability frameworks.
 - Reimbursement could include per-member-per-month payments or shared-savings attribution within value-based programs, such as those authorized under SSA §1899 and CMS Innovation Center Authority at SSA § 1115A.
 - Small practices could be offered bonus points towards the Promoting Interoperability performance category or final score in the MIPS program to provide a further incentive for investment in AI.

5. **Ongoing Reimbursement for and Valuation of AT Tools:** CMS’s reimbursement methodology for AI tools under Medicare, via the PFS, is particularly impactful because other payers, such as commercial payers and Medicaid, often look to Medicare to inform their own reimbursement methodologies.¹⁹ Undervaluing AI tools in

¹⁴ Centers for Medicare & Medicaid Services. “Technology-Enabled Care and Artificial Intelligence.” Available [Here](#).

¹⁵ Centers for Medicare & Medicaid Services. “Artificial Intelligence at CMS.” Available [Here](#).

¹⁶Centers for Medicare & Medicaid Services. “Resource Library.” Available [Here](#).

¹⁷ Department of Health and Human Services. “Artificial Intelligence (AI) Strategy.” Available [Here](#).

¹⁸ Centers for Medicare & Medicaid Services. “ACCESS Technical Frequently Asked Questions.” Available [Here](#).

¹⁹ Longyear R and Berenson R. “Artificial Intelligence Payment Policies: Challenges For CMS And The Medicare Physician Fee Schedule”. *Health Affairs*. January 2026. Available [Here](#).

the PFS may have a ripple effect on practices' ability to use AI tools in clinical care, especially when coupled with COA's longstanding concerns that the PFS does not keep pace with inflation.²⁰

AI tools differ from many of the services CMS currently reimburses because they are scalable, do not have physical limitations on use, and automate work.²¹ AI tools also have different pricing models that may be more difficult to capture when estimating costs – for example, AI tools may use an implementation fee, a subscription-based cost, a license fee at the user level, a per administration fee, or a combination of these fees.²² As new AI tools are being developed and existing tools continue to be updated, the underlying payment methodology for AI tools may need to be reevaluated more frequently.²³

Alongside reimbursement, providers need the operational tools to submit claims for AI tools. The American Medical Association (“AMA”) has developed new Current Procedural Terminology (“CPT”) codes that include AI (AMA refers to AI as “augmented intelligence”). Ongoing updates to this code set, along with coverage for and recognition of these codes, are necessary to continue accelerating the adoption of AI in clinical care.²⁴

Question 6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Tools that provide interactive clinical decision-making support have the potential to improve health care outcomes, provide new insights into quality, and reduce costs. For example, treatment guidelines, such as the National Comprehensive Cancer Network (“NCCN”) Guidelines, are a key resource for evidence-based and consensus-driven cancer care. These guidelines are updated at least annually by independent expert panels and play a critical role in the oncology space because they are often used by payers to determine coverage, and by providers to inform treatment pathways. However, these guidelines are typically published as static PDFs.²⁵

More dynamic implementations – such as interactive decision trees or AI-enabled query tools – have the potential to improve access to guideline-based recommendations at the point of care. When thoughtfully designed, such tools could support a more consistent application of evidence-based medicine while reducing cognitive burden on clinicians.

AI tools aimed at reducing administrative burden – such as ambient documentation, chart summarization, coding assistance, and patient identification for clinical trials – have shown promise in improving clinician experience and operational efficiency. Several community oncology practices have piloted these tools with encouraging qualitative results.

For example, one COA member practice has onboarded about 500 doctors using an AI scribe, which provides chart summarization and ICD-10 coding suggestions. The practice also uses AI to identify patients for clinical trials, who might otherwise be overlooked, through a pilot that assesses patient eligibility and screens them for suitability, ultimately providing more patients with access to clinical trials. Talking to a patient about a clinical trial can take four times as long as talking to a patient about standard of care therapy, so having these AI tools provides an opportunity to increase clinical trial access. Importantly, the AI scribe provides clinical decision support and does not supplant providers' autonomous decisions.²⁶

²⁰ <https://mycoa.communityoncology.org/publications/comment-letters/coa-comments-on-cy-2026-pfs>

²¹ Longyear R and Berenson R. “Artificial Intelligence Payment Policies: Challenges For CMS And The Medicare Physician Fee Schedule”. *Health Affairs*. January 2026. Available [Here](#).

²² Ibid.

²³ Ibid.

²⁴ O'Reilly K. “288 new CPT codes cover digital health, AI and more.” American Medical Association. 12 September 2025. Available [Here](#).

²⁵ Mullangi et al. “AI to Support Modern Cancer Care—The Augmented Oncologist.” *JAMA Oncology*. 4 September 2025. Available [Here](#).

²⁶ Patt D. “How AI Is Reshaping Oncology Practice: Insights From ASCO's AI Task Force.” 12 June 2025. Available [Here](#).

These are only a few examples of the many ways in which AI has the potential to support cancer care – applications also include cancer detection, screening, diagnosis, tumor analysis, biomarker assessment, and drug discovery.²⁷ Cancer researchers continue to investigate new use cases for AI, expanding the potential for its adoption in clinical care.²⁸

That said, most implementations to date remain early in maturity, and widespread evidence demonstrating measurable cost savings or improved outcomes remains limited. Emerging evidence suggests that the financial ROI for these tools remains modest and highly variable, particularly in community-based practice settings.

Recent peer-reviewed literature supports this observation. A 2024 study from the University of California, San Francisco, published in *JAMA Network Open*, evaluated the impact of ambient AI scribes on clinician productivity and billing, finding improvements in documentation and work relative value units (“RVUs”). The study did not find evidence of an increase in billing denials with AI scribe adoption.²⁹ However, the resulting financial gains may be insufficient to generate substantial net ROI to support practice implementation after accounting for licensing, implementation, and operational costs.

Similarly, an analysis from Kaiser Permanente published in *The New England Journal of Medicine* highlighted both the promise and limitations of ambient AI tools, noting that while clinician experience may improve, financial and operational returns remain uncertain and highly context dependent.³⁰

These findings align with the experience of many community oncology practices, including COA members, where AI pilots have demonstrated technical feasibility and quality-of-life improvements but have not yet produced consistent or scalable financial returns.

Collectively, these experiences suggest that while AI holds significant potential to improve care delivery, most current applications are still in an early stage of development. Real-world implementation challenges observed in community oncology must be addressed to facilitate sustained adoption. Across multiple AI pilot programs conducted by community oncology practices, several consistent themes have emerged:

- **Data quality and preparation remain significant barriers:** AI tools that rely on clinical documents, scanned records, or faxes require extensive human oversight and data normalization. Even well-performing tools often require prolonged quality assurance efforts to achieve acceptable accuracy. In practice, accuracy rates approaching, but not reaching, 100 percent remain common, limiting the extent to which automation can replace manual review.
- **Operational lift is frequently underestimated:** The effort required to vet vendors, negotiate contracts, implement integrations, train staff, monitor performance, and manage ongoing quality control is substantial. These indirect costs are often not reflected in vendor ROI projections.
- **Quality-of-life improvements do not always translate into financial ROI:** Many AI tools deliver value through reduced cognitive burden, improved clinician satisfaction, or streamlined workflows. While meaningful, these benefits do not always translate into measurable cost savings or revenue generation – particularly in fee-for-service or hybrid reimbursement environments.

²⁷ Luchini C, et al. “Artificial intelligence in oncology: current applications and future perspectives.” *British Journal of Cancer*. 26 November 2021. Available [Here](#).

²⁸ Pancholi N. “Eye on AI: Applying Artificial Intelligence to Drive Cancer Research, Part 2.” American Association for Cancer Research. 18 August 2025. Available [Here](#).

²⁹ Holmgren AJ, et al. “Ambient Artificial Intelligence Scribes and Physician Financial Productivity.” *JAMA Network*. 9 January 2026. Available [Here](#).

³⁰ Tierney A et al. “Ambient Artificial Intelligence Scribes to Alleviate the Burden of Clinical Documentation.” 21 February 2024. *NEJM Catal Innov Care Deliv*. Available [Here](#).

- **Voice and patient-facing AI tools remain immature:** Early pilots of conversational or voice-based agents have struggled to meet both patient expectations and clinical workflow needs. Aligning patient intent, clinical appropriateness, and operational efficiency remains challenging, and few implementations have demonstrated sustained value.
- **Revenue cycle applications show promise but face trust barriers:** AI tools aimed at coding, prior authorization (“PA”), or denial management may eventually generate measurable savings; however, most organizations are not yet comfortable reducing staffing levels without prolonged validation periods. As a result, anticipated financial efficiencies have not yet been realized.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers generally support the use of technology that improves access, reduces delays, and enhances coordination of care. At the same time, they value transparency, strong privacy protections, and continued human clinical oversight. Policies that reinforce these expectations will be essential to sustaining trust in the use of AI in clinical care.

1. **Concerns Regarding AI Use in Utilization Management, Including Prior Authorization**

AI use in utilization review is of great concern to COA. We have long expressed concerns about how PA places a significant burden on providers and patients.^{31,32} A survey of physicians found that 94 percent of physicians surveyed stated that PA had a “somewhat or significant negative impact” on patient clinical outcomes, and more than a quarter of physicians reported that PA led to a serious adverse event for a patient in their care. Sixty-one percent of physicians surveyed indicated that they are concerned that AI increases or will increase PA denial rates.³³ In turn, denials delay patient access to treatment, which is particularly pressing for patients with cancer, as delays in treatment initiation have been associated with increased mortality across several cancer types.³⁴

These findings underscore the risks of AI use in an immensely burdensome process that adversely impacts patient care, particularly as insurers’ use of AI grows. A survey from the National Association of Insurance Commissioners (“NAIC”) found that 84 percent of responding health insurers use AI/machine learning.^{35,36} Utilization management is a primary use, with 71 percent reporting they currently or are exploring using AI for utilization management practices.³⁷ While we acknowledge that AI may speed up utilization review, providers and patients need confidence that wrongful denials will not occur because of a lack of human oversight. Unfortunately, at this time, many patients and providers are not even aware in cases where AI is involved in utilization review; NAIC’s survey found that less than a quarter of insurers tell providers when AI is used.³⁸

While CMS has taken some steps to regulate Medicare Advantage organizations’ (“MAOs”) use of AI in making coverage determinations and has stated that it is the responsibility of the MAO to ensure that AI complies with

³¹ Community Oncology Alliance. “COA Comments on WISER Model,” 14 August 2025. Available [Here](#).

³² Community Oncology Alliance. “COA Comments on 2027 Medicare Advantage and Part D Proposed Rule,” 23 January 2026. Available [Here](#).

³³ American Medical Association. “2024 AMA prior authorization physician survey,” February 2025. Available [Here](#).

³⁴ American Cancer Society Cancer Action Network (ACS CAN). “The Costs of Cancer: Addressing Patient Costs.” 2020. Available [Here](#).

³⁵ Mello M et al. “The AI Arms Race In Health Insurance Utilization Review: Promises Of Efficiency And Risks Of Supercharged Flaws.” January 2026. Available [Here](#).

³⁶ DeFrain K et al. “Health Insurance Artificial Intelligence/Machine Learning Survey Results.” National Association of Insurance Commissioners (NAIC). 2025. Available [Here](#).

³⁷ DeFrain K et al. “Health Insurance Artificial Intelligence/Machine Learning Survey Results.” National Association of Insurance Commissioners (NAIC). 2025. Available [Here](#).

³⁸ DeFrain K et al. “Health Insurance Artificial Intelligence/Machine Learning Survey Results.” National Association of Insurance Commissioners (NAIC). 2025. Available [Here](#).

CMS's requirements for how coverage determinations are made, we are concerned that these regulations may provide insufficient beneficiary protections.^{39,40} HHS Office of Inspector General ("OIG") recently issued an updated Medicare Advantage Industry Segment-Specific Compliance Guide that echoes concerns regarding the use of AI in PA and coverage decisions.⁴¹ However, patient protections must expand across the health care sector. For example, in a recent Medicaid and CHIP Payment and Access ("MACPAC") session, staff found that federal oversight of automation in AI is especially limited in Medicaid.⁴²

To add further guardrails regarding plans' use of AI and to promote transparency, we recommend CMS expand PA reporting requirements to require health plans to also report when AI was used in PA decisions, stratified at the item or service level for both drugs and services, with a breakdown of the percentages of whether requests were approved or denied. CMS is already implementing public reporting requirements for PA of services, but these are likely to mask trends in PA decisions since they are in the aggregate and are for services only.⁴³ We believe that data collection and public reporting from health plans that use AI in coverage decisions are vital for providers, patients, and CMS, and support strategies that facilitate the collection of accurate, transparent data. However, increased transparency and data collection are not a substitute for policies that make the role of human oversight clear.

Recent research from Stanford University highlights several concerns and risks related to the use of AI: doubts regarding the thoroughness of human review for AI-made decisions; users' lack of familiarity with AI and how it works; lack of information regarding what drives an AI prediction; underperformance, where models miss the mark; unintended consequences, such as the creation of perverse incentives related to denials; and variance in governance practices across health care organizations.⁴⁴

While all of these issues are of significant concern, COA is particularly worried about how AI's role in utilization management might impact the use of precision medicine, which plays a vital role in oncology. Precision medicine and targeted therapies enable physicians to tailor treatments to each patient's cancer based on its genetic and molecular characteristics. Restrictive insurance policies already often fail to keep pace with these innovations, and the use of AI in utilization management for these treatments may further delay patient access to needed cancer care.⁴⁵

2. **Concerns Regarding Patient Privacy and Security**

Patient privacy and security are also critical in facilitating AI adoption. HHS itself has reported a 256 percent increase in large breaches involving hacking and a 264 percent increase in ransomware over five years, making data and security concerns particularly pressing.⁴⁶ Patients must be able to trust that their information will be used in a HIPAA-compliant manner, that they will be informed of when their data will be used within an AI tool, and that safeguards are in place in the event of a data breach.

³⁹ Centers for Medicare & Medicaid Services. "Frequently Asked Questions related to Coverage Criteria and Utilization Management Requirements in CMS Final Rule (CMS-4201-F)." Available [Here](#).

⁴⁰ Centers for Medicare & Medicaid Services. "Medicare Program; Contract Year 2024 Policy and Technical Changes to the Medicare Advantage Program, Medicare Prescription Drug Benefit Program, Medicare Cost Plan Program, and Programs of All-Inclusive Care for the Elderly." 12 April 2023. Available [Here](#).

⁴¹ Department of Health and Human Services Office of Inspector General. "Medicare Advantage Industry Segment-Specific Compliance Program Guidance." February 2026. Available [Here](#).

⁴² Medicaid and CHIP Payment and Access Commission. "January 2026 Public Meeting." 29-30 January 2026. Available [Here](#).

⁴³ Centers for Medicare & Medicaid Services. "Medicare and Medicaid Programs; Patient Protection and Affordable Care Act; Advancing Interoperability and Improving Prior Authorization Processes for Medicare Advantage Organizations, Medicaid Managed Care Plans, State Medicaid Agencies, Children's Health Insurance Program (CHIP) Agencies and CHIP Managed Care Entities, Issuers of Qualified Health Plans on the Federally-Facilitated Exchanges, Merit-Based Incentive Payment System (MIPS) Eligible Clinicians, and Eligible Hospitals and Critical Access Hospitals in the Medicare Promoting Interoperability Program." 8 February 2024. Available [Here](#).

⁴⁴ Mello M et al. "The AI Arms Race In Health Insurance Utilization Review: Promises Of Efficiency And Risks Of Supercharged Flaws." *Health Affairs*. January 2026. Available [Here](#).

⁴⁵ Community Oncology Alliance. "COA Position Statement on Physician Autonomy and Patient Access to Cancer Treatment." 24 March 2025. Available [Here](#).

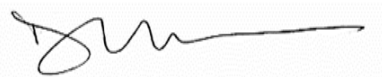
⁴⁶ Department of Health and Human Services Office for Civil Rights. "HHS Office for Civil Rights Issues Letter and Opens Investigation of Change Healthcare Cyberattack." 13 March 2024. Available [Here](#).

Conclusion

Structured incentives tailored to support community practices through the adoption of AI are vital for fostering innovation, ensuring access to advanced treatments, and maintaining the competitive edge necessary to thrive in an evolving health care landscape. We propose that these incentives be modeled after successful strategies used during the EHR rollout, with the aim of encouraging engagement and investment in AI integration across the board. A multi-faceted approach, focusing on financial incentives and infrastructure development, training and partnerships, regulatory streamlining, and reimbursement, is critical to accelerate AI adoption. At the same time, policies that expand AI use must incorporate transparency, strong privacy protections, and continued human clinical oversight.

Thank you for your request for information and consideration of these comments. COA is available to discuss any of the concerns or recommendations in this letter and look forward to working with you on promoting the health and well-being of Americans with cancer.

Sincerely,

A handwritten signature in dark ink, appearing to read 'Debra Patt', with a long horizontal flourish extending to the right.

Debra Patt, MD, PhD, MBA, FASCO
President

A handwritten signature in dark ink, appearing to read 'Ted Okon', with a long horizontal flourish extending to the right.

Ted Okon, MBA
Executive Director